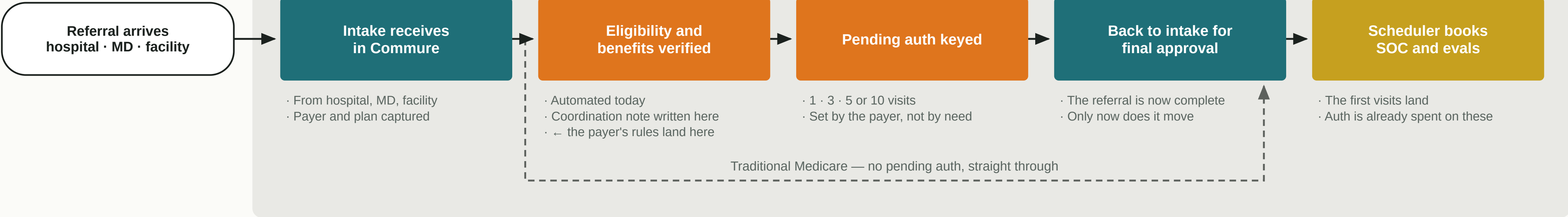


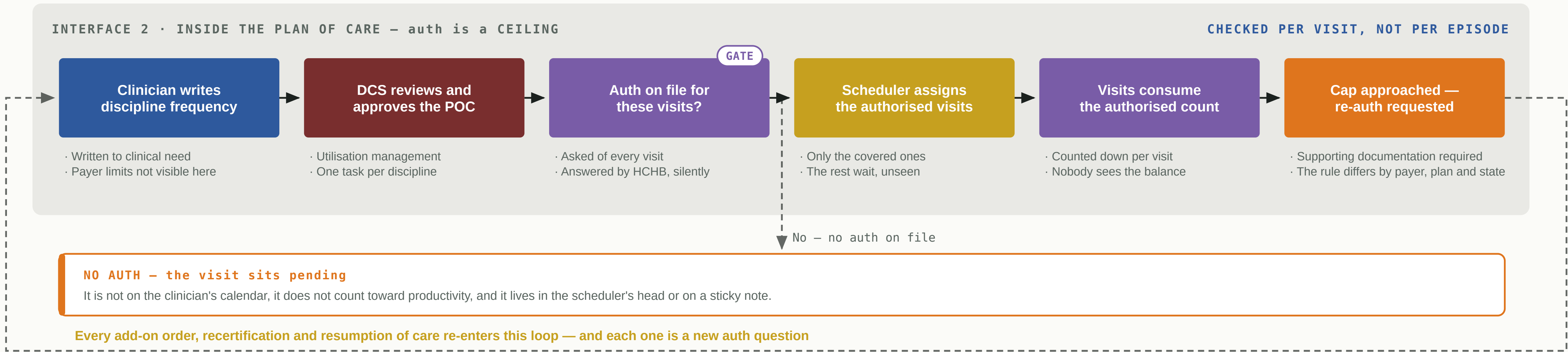
Authorization

Two interfaces only — the gate before the first visit, and the ceiling inside the episode

TRIGGER



“We know we have the referral, but it is just not in my workflow to schedule yet — it is stuck in auth.”



WHAT THE PAYER HAS ALREADY DECIDED — before anyone writes a plan of care

THE CAP IS SET UPSTREAM

- Every payer, plan and state sets its own — there is no single rule to learn
- Indiana Medicaid — 8 visits shared across PT / OT / ST
- Indiana Medicaid — 30 days from the discharge date, not the admit date
- Ohio Medicaid — caps similarly; commercial and MA plans differ again

TWO CEILINGS, NOT ONE

- Auth is permission — how many visits the payer will allow
- PDGM is payment — the economic band for the period
- LUPA is the floor; utilisation management is the ceiling
- A visit can be authorised and still be uneconomic

The data already exists. The auth team writes the payer's rules into a coordination note at verification — days before anyone writes the plan of care.

Surfacing those rules at plan-of-care creation is the highest-value, lowest-complexity win in this flow — and a patient-care win, because abrupt discharges happen when nobody planned for the real visit budget.

BOUNDARIES

Auth is drawn only where it meets scheduling

Traditional Medicare has no pending auth

Auth governs permission · PDGM governs payment

WHERE IT BREAKS

Pending-auth visits are invisible not on a calendar, not counted

~50 pending-auth workflows a day so schedulers bulk-clear without reading

Plans of care ignore payer limits then the team is surprised

Abrupt discharge when the visit budget quietly runs out